

PSYCHIATRY

Abstract & Creative Thinking

A crisp guide to daily practice, clinical assessment, and interpretation

DEFINITION

PRACTICE

ASSESSMENT

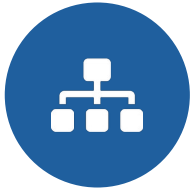
From “What is it?” to “What does it mean?”

Educational presentation — not a standalone diagnostic test



Definition: two thinking skills

The first distinction makes the rest of the topic easier to understand.



ABSTRACT THINKING

Moving from specific facts to general meaning, relationships, categories, and principles.

Think: "What is the deeper meaning?"



CREATIVE / DIVERGENT THINKING

Generating multiple possible ideas, uses, explanations, or solutions — then exploring or improving them.

Think: "What else could be possible?"

KEY IDEA

Abstract thinking finds the pattern. Creative thinking expands the possibilities.

QUESTION 1

How can I improve abstract thinking?

Train yourself to move from the concrete example to the underlying idea.



01 Proverbs & metaphors

Find the deeper meaning, then make your own example.



02 Find similarities

Compare unrelated things and identify the higher-level link.



03 Categorization

Group the same items at different levels of abstraction.



04 Why? / What if?

Ask why a pattern occurs and what changes if one part is altered.



05 Explain without the word

Describe the concept without naming it.



06 Read → summarize

Reduce a paragraph to one sentence: the main idea.

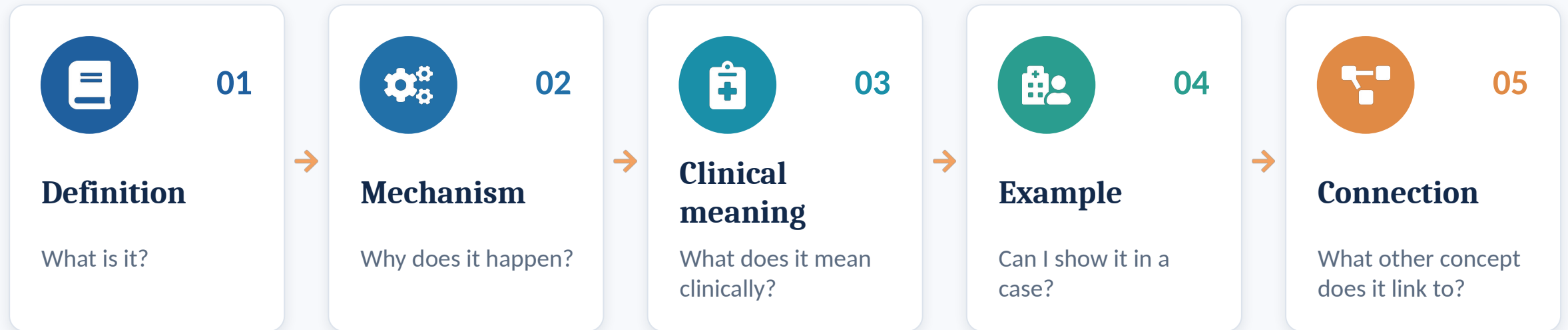


DAILY TARGET

10 minutes: one concept, one deeper meaning, one new connection.

Use one clinical concept every day

A simple ladder keeps the practice clinically relevant.



EXAMPLE

“Depression” → define it → ask why symptoms occur → interpret function and impact → give a case → connect to sleep, cognition, and risk.

Practice the reasoning, not the wording.

How can I improve creative thinking?

The core habit is to produce more than one possible idea before judging them.

PRACTICAL EXERCISES

-  **Alternative uses:** list 10 uses for one ordinary object.
-  **What if?:** explore unusual consequences and possibilities.
-  **One problem, 3 ways:** force yourself to find three solutions.
-  **Combine unrelated things:** merge functions from two objects.
-  **Three endings:** change the outcome of the same scenario.
-  **5-minute brainstorm:** quantity first, judgment later.

EASY RULE

-  **Generate**
-  → **Explore**
-  → **Combine**
-  → **Improve**

 **Best live demo:** give the class a paper clip and ask for as many uses as possible in 60 seconds.

As a doctor, how can I test these skills?

For a Mental Status Examination, abstract thinking can be assessed clinically; creative thinking is better explored as flexibility and idea generation.



Use simple prompts that test generalization, similarity, and multiple ideas — not a single “right” answer.



ABSTRACT THINKING

Proverb meaning + similarity question



CREATIVE / DIVERGENT THINKING

Alternative-use task + follow-up prompt

Interpret with context

Consider education, culture, language, attention, mood, and the rest of the MSE before drawing conclusions.



Education



Culture



Language



Attention



Mood



Rest of MSE

Testing abstract thinking

Short, natural questions can reveal whether the patient moves beyond literal or concrete responses.



DOCTOR – PATIENT



Doctor

“What does ‘Don’t judge a book by its cover’ mean?”

Patient

“We should not judge a person only by their appearance.”



Doctor

“How are a dog and a cat alike?”

Patient

“Both are animals and can be kept as pets.”



CONCLUSION

Preserved / intact abstract thinking

Why?

The patient identifies a general meaning and a higher-level similarity rather than staying only with concrete details.

Exploring creative / divergent thinking

Look for variety, flexibility, and the ability to generate alternatives.



DOCTOR - PATIENT



Doctor

"Imagine I give you a paper clip. Tell me as many uses as you can."

Patient

"To hold papers, as a bookmark, to organize wires, or for a small craft."



Doctor

"Can you think of another unusual use?"

Patient

"I could use it as a temporary zipper pull."



CONCLUSION

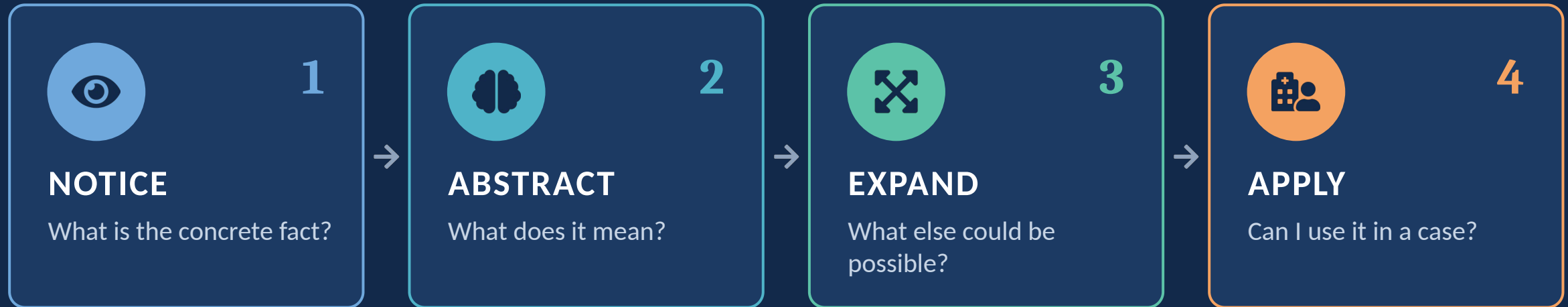
Flexible, varied idea generation

What to notice

- More than one possible idea
- Ability to shift or add alternatives
- No need to reward "cleverness" alone

Use the skills in a simple cycle

A memorable closing framework for study and clinical practice.



Clinical reminder

One response is not a diagnosis. Interpret thinking style with education, culture, language, attention, mood, and the rest of the Mental Status Examination.

Core message: train the shift from facts → meaning → possibilities → clinical application.

PSYCHIATRY

Thank You

Questions & discussion

Facts → Meaning → Possibilities → Clinical application

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